

23rd.—Twelve A.M.: No abatement of the spasms; still increasing. Strength good; pulse 84. Repeat enema, and to take the following pills: Extract of cannabis indicus, six grains; extract of hyoscyamus, and assafoetida, of each half a drachm: divide into twelve pills; one every hour.

24th.—Ten A.M.: Bowels had not acted; no abatement of spasms. To take tincture of Indian hemp, thirty drops in water every two hours; and repeat the enema.

25th.—Bowels acted; spasms still increasing; pulse 72, and much weaker; skin cold and perspiring; tongue clean. Wine and beef-tea to be taken every two hours, and the Indian hemp continued. Counter-irritation to the spine with croton oil and turpentine.—Five P.M.: Much improved; pulse 84, and stronger.

26th.—Nine A.M.: Spasms still continue, but more confined to the centre of the spine; pulse 80, firm and strong; each paroxysm to-day is ushered in with a cry of pain referred to the centre of the back; expression of face bad—"that peculiar expression noticed in tetanus." Continue the Indian hemp, increased to forty minims; and beef-tea and wine.—Five P.M.: Much worse. The muscles of the whole system were now more or less involved with spasm, and trismus was now complete. While I was standing by the bed-side, he was seized with a severe fit, the respiratory muscles being affected to such a degree as to prevent him from breathing; the jaw fixed and rigid, the eyes fixed, and the face fearfully distorted; respiration was suspended, the heart ceased to pulsate, and he was apparently suffocated. In a few minutes there was relaxation of the muscles, but no return of the heart's action or respiration: he was to all appearance dead. Something must be done to try to bring about reanimation. I put in operation Marshall Hall's method of treatment. In about five minutes there was a deep inspiration, followed soon after by pulsation of the heart. The treatment was continued for some time, until he was quite restored. There was no return of spasm with the reanimation. Continue the Indian hemp, beef-tea, and wine; and the following draught to be taken immediately: solution of hydrochlorate of morphia, forty-five drops; tincture of hyoscyamus, forty-five drops; camphor mixture, one ounce and a half.—Eight P.M.: Much better, having slept for several hours; slight return of spasms in the back on awaking.

27th.—Ten A.M.: Had spent a good night; slept well; pulse 84, full and strong; expression of face natural; makes no complaint of pain; no return of trismus; no difficulty in swallowing.—Three P.M.: Still improving. Continue the Indian hemp, beef-tea, and wine. As the bowels had not acted, he was ordered to take the following draught immediately—five grains of calomel, and half a grain of hydrochlorate of morphia; also a purgative enema in the morning.

28th.—Ten A.M.: Bowels acted twice; much improved; pulse 76, full and strong; tongue clean; still slight spasmodic twitches in the muscles of the spine about every hour, but no pain.

29th.—Eleven A.M.: Still improving; pulse 84, full and strong; tongue clean; no tenderness on pressure over the abdomen or spine; spasms of back continue, though slight. About seven P.M. I received a message to visit him, as he had been very restless during the afternoon. "His residence being about two miles' distance, and not an urgent message to visit him immediately," I arrived there in the course of an hour, and found him quite dead. In attempting to get out of bed, he was seized with spasm of the entire body, involving the respiratory muscles, and died in less than two minutes. He was conversing with his wife freely about five minutes before this sudden return of spasm, and said that he felt much better and quite free from pain. There was no chance of reanimation, as he had been dead nearly an hour. No post-mortem examination of the body was made.

There was no prostration of nervous power to cause death. The most prominent symptoms had subsided—namely, muscular rigidity and spasm. The pulse was good, all expression of suffering gone, and sleep restored. Spasm of the respiratory muscles and heart could be the only cause of this sudden death.

CASE 2.—In 1857, I was requested to visit the son of a medical man, aged twelve years. He had been complaining of stiffness of the muscles of the neck and lower jaw for two or three days, attributed to a common cold, arising from being overheated when playing at cricket, and afterwards sitting on the wet grass. When I saw him, the spasm had extended to the muscles of the back, abdomen, and extremities, with great pain in the contracted muscles; pulse and respiration quick, with complete trismus. This latter symptom continued for six

or seven days, without any relaxation of the muscles of the lower jaw.

The treatment was the same as in the former case: tincture of Indian hemp, ten minims every two hours, increased to twenty minims; calomel occasionally at night, and enema on the following morning; beef-tea and wine, with counter-irritants to the spine. The spasm and rigidity of the muscles were so great in this case, that when the little fellow was raised up he was as rigid as a block of wood. All his medicine and nourishment were sucked through an opening between the teeth with a small tube. The spasms partially abated after the seventh day, and he quite recovered. The only additional treatment was during the convalescence, a little citrate of iron and quinine.

Melbourne House, Hampstead, Ang. 1860.

A Mirror OF THE PRACTICE OF MEDICINE AND SURGERY IN THE HOSPITALS OF LONDON.

Nulla est alia pro certo noscendi via, nisi quam plurimas et morborum et dissectionum historias, tam aliorum proprias, collectas habere et inter se comparare.—MORGAGNI. *De Sed. et Caus. Morb.*, lib. 14. Proœmium.

ST. GEORGE'S HOSPITAL.

VACCINATION, FOLLOWED BY INFLAMMATION AND ABSCESS
OF THE ARM ON THE RECEIPT OF A BLOW UPON
THE SCAB; FATAL PYÆMIA.

(Under the care of Mr. CÆSAR HAWKINS.)

THE phenomena detailed in the following case are of such an unusual character that we feel disposed to set them down as the result of some inherent vice in the constitution of the patient. A rather severe blow received upon the dried-up vesicle of vaccination was followed by extensive inflammation and abscess, of so serious a nature as to end in pyæmia and death. It is, of course, a common occurrence for an accidental blow or injury to be received on the vaccine vesicle of a child, and to be followed, perhaps, by a certain amount of inflammation; but seldom indeed has the mischief proceeded so far as in the present instance. We have also known sloughing to ensue around the vesicle, without preceding injury to the part. Dr. Gregory mentions, in his "Practice of Medicine," that in some instances the specific inflammation or areola is very violent, extends from the shoulder to the elbow, invades the trunk of the body, and requires to be assuaged by cold lotions; but this, we may observe, is one of the irregularities in the phenomena of vaccination. We place the present case on record, however, from its extreme rarity in respect of the severity of the symptoms which supervened. A point of interest in the post mortem examination was the detection of air in the pericardium.

Charlotte W.—, aged thirty-five, admitted March 31st. One month previously she was vaccinated; the vaccine took well, but when the pustule had scabbed over she gave her arm a violent blow; this was followed by inflammation of the arm. On admission, the upper part of the left arm was very red, swollen, and painful, and there was threatened suppuration at the upper and back part. She looked pale and thin, and was given meat and porter. The swelling at the back of the arm increased, and on the 9th of April Mr. Hawkins opened an abscess in that situation. It appeared to extend across the back part of the shoulder-joint and under the pectoral muscles. A large amount of thick pus escaped.

April 14th.—She had a rigor which lasted half an hour. Complained of much pain in her arm; the abscess was discharging freely. Tongue loaded; pulse 84, weak. She was given an ounce of brandy and a saline mixture with ipecacuan (five grains) every six hours.

15th.—The countenance was anxious; she was constantly

sweating during the night; and complained of pain, which came on suddenly, over the right patella.

On the 16th the bursa patellæ was found to be inflamed, very tender, and fluctuating. She was very sick during the day. The abscess was discharging considerably; the pus was unhealthy, and had extended beneath the scapular portion of the deltoid. The bursa was opened, and pus mixed with bursal fluid escaped.

17th.—Another rigor took place, leaving her in a very exhausted condition; there was a slight erysipelatous blush over the arm.

The erysipelas continued to spread, and on the 21st the whole body and extremities were implicated. The discharge from the abscess was very offensive, and mixed with blood; pulse 140. There were several gangrenous patches situated over the prominent parts of the ribs and right side, and also over the right nates. From this time she gradually sank. She was given a pint of wine daily, with as much beef-tea as she could take; but she showed no disposition to rally, and died on the 23rd, at seven A.M.

Examination thirty hours after death.—The body was in good condition. There was a large abscess in the left axilla, which had been opened during life; it contained rather foul pus. The marks of vaccination on the left arm were very faintly perceptible. There was an abscess over the right patella, which had also been opened; the joint was normal. The pericardium contained a large quantity of air; in other respects it was natural. Heart, lungs, and other viscera healthy. The uterus was retroverted, and lay supported by the rectum, pointing towards the left sacro-ischiatric foramen. Its posterior wall contained two very small fibrous tumours. The ovaries contained each a false corpus luteum. They, as well as the uterus, were natural. No morbid appearances were found in the great veins of the trunk and thorax; those of the limbs were not dissected, in consequence of the objections of friends.

UNIVERSITY COLLEGE HOSPITAL.

ACUTE INFLAMMATORY OEDEMA OF THE SCROTUM, FOLLOWED BY EXTENSIVE SLOUGHING OF THE INTEGUMENTS, PYÆMIA, AND DEATH.

(Under the care of Mr. ERICHSEN.)

MANY years ago, Mr. Liston described (in the twenty-second volume of the "Medico-Chirurgical Transactions") an affection of the scrotum, which he characterized by the name of "Acute Anasarca of the Scrotum." This part of the body becomes suddenly distended by the effusion of serous fluid, the disease resembling, in many of its characters, extravasation of urine; but its treatment is somewhat different from that of the latter affection.

"This acute œdema often follows on sores or eruptions situated in the groin, genitals, or inside of the thighs, and on fistulæ about the perinæum and anus. These may, perhaps in consequence of some constitutional disturbance, have the secretion from them suppressed, and they may become surrounded by a slight erythematous blush. Suddenly the scrotum begins to swell, and sometimes in less than twelve hours it will be found immensely distended; and this arises generally from the infiltration apparently of a very acrid and fetid sanies. If this be allowed to distend the scrotum, without being interfered with, it will have the same bad effects as the effusion of urine. On inquiry it will generally be found that there has existed no previous disease of the urethra, or impediment to the passage of the urine, and a large-sized catheter may be put into the bladder without meeting with any obstruction."—*Liston's Practical Surgery*, p. 450.

As the disease advances, unless early and energetic treatment be adopted, the scrotum becomes hot and shining, the integument sloughs, and the whole cellular tissue of the part is destroyed. The treatment consists, at the commencement, in a free incision in the mesial line, or on each side of the raphé; fomentations and support to the parts. If sloughing occurs, the ordinary principles of treatment must be adopted.

Such an instance of disease has just terminated fatally in the above hospital. (We are indebted to Mr. H. C. Bastian, house-surgeon, for the notes of the case.) The attack came on without any assignable cause, and succeeded rapidly on a slight pain during micturition. The scrotum swelled to the size of an adult's head; and although free incisions were made, the whole of the scrotal integument sloughed, and was removed in

one single mass with a pair of dressing forceps, leaving the testicles quite bare. Granulations sprang up on the testicles, and appearances seemed favourable, when the left groin and thigh began to swell, pyæmia became developed, and the patient died. It is a good sample of a rare disease, first described by Mr. Liston, and is occasionally witnessed in hospital practice. The symptoms, at first sight (as Mr. Erichsen clinically observed), were not unlike urinary extravasation; but the history was clear of absence of stricture or other impediment to the flow of urine; and he considered it an attack of acute idiopathic phlegmonous erysipelas of the scrotum. When the slough was detached, the man's condition was so good as to lead to a hopeful prognosis, particularly as there was no indication of the mischief higher up, nor any tendency to involve the peritoneum.

Under the heading of Diffuse Inflammation of the Scrotum, Mr. Curling* speaks of this affection as occurring under two forms. In one, he observes, it is mild and unattended with danger, and terminates favourably under gentle antiphlogistic treatment. In the other form (of which the following is an example), the complaint is severe and dangerous, and prompt and decisive measures are requisite to avert serious consequences.

John T—, aged twenty-eight, ironmonger, single, admitted Aug. 17th, 1860. Habits regular and temperate; diet good; has generally enjoyed excellent health, but about six years ago contracted a gonorrhœa. He has never had stricture, nor the slightest difficulty in micturition, and instruments have never been passed into his urethra. A few days after the commencement of the gonorrhœa he noticed a small and hard swelling in the perinæum, apparently connected with the urethra, and also scalding in this situation when he passed urine; at the same time the penis became œdematous and swollen to more than twice its natural size, so that the medical man who attended him was obliged to make incisions into it, which gave exit to an oily-looking and very offensive discharge. As the penis increased in size, the swelling in the perinæum gradually subsided of its own accord.

Since this first attack, six years ago, the penis has swelled up six or seven times at pretty regular intervals, being preceded each time by the lump in the perinæum and pain in micturition, lasting about a week or ten days, and then bursting on the anterior and under surface of the penis, and giving exit to the same foetid discharge as before. Each of these attacks has supervened without any assignable cause. The patient feels quite confident they were not due to fresh "claps," and sometimes they have come on when he has had no connexion for six weeks or two months previously.

The present attack commenced on the 14th of August, without any apparent cause, by a slight pain during micturition in the perineal region, and then a swelling, which soon extended to the scrotum; but the penis did not become affected till the 16th. On the day before this the medical man in attendance had made several punctures in the scrotum, and let out some serous fluid; but when seen by Mr. Erichsen on the evening of the next day the scrotum and penis were enormously distended, the former being about the size of a man's head, and sloughs had already formed in the lower and posterior parts. Free incisions, about six inches in length and one and a half in depth, were at once made on each side of the raphé, and exit given to a quantity of brownish fluid; poultices were then ordered to be applied to the part, and the patient was well supported with brandy.

The next day he was admitted into the hospital, the penis and scrotum being still greatly distended, though somewhat diminished in size. The inflammation had extended anteriorly to the upper part of the thighs, involving the lower part of the abdomen, and were covered with an erysipelatous blush. Pulse 155, full; tongue white, furred. Two more incisions were made into the scrotum, and the patient ordered to have chlorinated poultices applied to it, and hot fomentations to the thighs and abdomen; two ounces of egg-and-brandy mixture every three hours, and ten grains of carbonate of ammonia, with a drachm and a half of tincture of cinchona in camphor mixture, every six hours; meat diet, two pints of beef-tea, and one pint of milk.

Aug. 18th.—Erysipelas diminished; pulse 120, not so strong; tongue less furred; penis a little more swollen; anterior and lower part of scrotum one mass of slough, but a line of demarcation has formed on the upper part, higher on the left than on the right side; micturition free and painless. To continue medicine as before, and scrotum to lie raised.

19th.—Erysipelas has disappeared; pulse 130, pretty full;

* On Diseases of the Testis, 2nd edit., p. 470.